

The Structural Realignment of Pennsylvania's Health Insurance Markets: A 2026 Actuarial and Policy Assessment of Rating Area 6 and the Lehigh Valley Crisis

The Pennsylvania individual health insurance market in July 2026 is experiencing its most severe structural disruption since the inception of the state-based exchange, Pennie. This destabilization is driven by the simultaneous convergence of a major federal subsidy contraction and double-digit gross premium rate hikes approved by the Pennsylvania Insurance Department. Across the Commonwealth, these forces have triggered historic enrollment attrition, leaving more than 160,000 formerly insured Pennsylvanians without coverage. While the macroeconomic subsidy contraction represents a universal threat across all nine of the state's geographic rating areas, its downstream consequences are highly localized. In Rating Area 6—which encompasses Lehigh County and the broader Lehigh Valley—the affordability shock has collided with a high-profile provider-insurer contract termination between UnitedHealthcare and Lehigh Valley Health Network. This analysis provides an evaluation of the statewide premium and enrollment dynamics, a comparative assessment of the regional rating areas, a detailed evaluation of the Lehigh Valley network rupture, and an assessment of the regulatory protections available to vulnerable consumers.

Macroeconomic Disruption: The Post-Enhanced Tax Credit Era

The severe premium inflation confronting Pennsylvania consumers in 2026 is the mechanical result of two distinct fiscal adjustments occurring in tandem: the expiration of the federal Enhanced Premium Tax Credits and the annual regulatory gross rate adjustments. The Enhanced Premium Tax Credits, originally enacted under the American Rescue Plan Act of 2021 and extended through December 31, 2025, capped individual premium contributions on a sliding scale and eliminated the historical subsidy eligibility cap at 400% of the Federal Poverty Level. Congress's failure to extend these enhancements into 2026 re-established the pre-2021 subsidy structure, introducing a steep financial income cliff for middle-class households. Concurrently, the Pennsylvania Insurance Department approved a statewide weighted-average gross rate increase of 21.5% in the individual market and 12.7% in the small-group market. While state regulators blocked \$50.1 million in carrier-proposed rate increases by correcting unsupported actuarial assumptions, the net effect of the gross rate increases combined with the lost federal tax credits resulted in a 102% average net premium increase for the subsidized marketplace population.

Carrier Name	Product Type	Requested Rate Change	Approved Rate Change	Actuarial and Underwriting Context	Mapped Rating Areas
Ambetter Health of Pennsylvania	HMO / EPO	30.1\%	37.8\%	Increased during regulatory review due to worsening risk pool morbidity.	Areas 3, 8
Capital Advantage Assurance Company	PPO / EPO	26.0\%	24.6\%	Adjusted downward on review; reflects regional clinical cost trend lines.	Areas 7, 9
Geisinger Health Plan	HMO	14.1\%	11.6\%	Actuarial assumptions corrected downward; strong integrated delivery network.	Areas 2, 5, 6
Jefferson Health Plans (HPP)	HMO	7.3\%	16.7\%	Adjusted upward during regulatory review to account for commercial risk pool instability.	Areas 3, 8
Highmark, Inc.	PPO / EPO	17.2\%	17.7\%	Aligned with regional clinical cost profiles and narrow-network product pricing.	Areas 1, 4, 6
Jefferson Health Plans (Partners)	PPO	-10.1\%	-10.1\%	Sole approved rate decrease in Pennsylvania; expansion of streamlined EPO networks.	Areas 3, 6, 8

The reintroduction of the strict 400\% Federal Poverty Level subsidy cliff has disproportionately harmed moderate-income households. Under the 2026 Internal Revenue Service contribution tables, households below the 400\% threshold are subject to a sliding contribution scale that tops out at 9.96\% of annual income, up from the flat 8.5\% cap under the expired rules. For households whose income crosses the 400\% Federal Poverty Level line by even one dollar, the

applicable premium tax credit falls immediately to 0%, leaving them solely responsible for the full gross premium.

The financial exposure of typical Pennsylvania households across various income bands illustrates the severity of this transition:

$\text{Required Premium Contribution} = \text{Applicable Percentage Table Rev. Proc. 2025-25} \times \text{Household Income}$

- **Median Single Worker (399.5% FPL):** A single full-time worker earning the median Pennsylvania salary of \$62,529 sits on the precise edge of the cliff. Under 2026 rules, their required annual premium contribution rose from \$5,315 (8.5%) to \$6,228 (9.96%). A minor cost-of-living adjustment or overtime payout will push this individual over the 400% Federal Poverty Level threshold, stripping them of all financial assistance and rendering the plan entirely unsubsidized.
- **Median One-Person Household (495.5% FPL):** A single-person household earning the median income of \$77,545 is entirely unsubsidized. They must transition from a capped annual premium of \$6,591 in 2025 to paying the full gross premium of the plan, which can exceed several thousand dollars per month depending on age and geographic rating area.
- **Median Two-Person Household (366.7% FPL):** A cohabitating couple with a combined median income of \$77,545 remains subsidized but faces an annual premium contribution increase of \$1,132, rising from \$6,591 to \$7,723.
- **Median Family of Four (311.9% FPL):** A family of four earning \$100,262 remains below the cliff but experiences an absolute cost increase of \$1,464 annually, with their required premium contribution scaling up to \$9,986.

This subsidy cliff's real-world impact is demonstrated by a prominent case study highlighted by state regulators: a 60-year-old married couple in York County earning approximately \$82,000 per year. Sitting at roughly 388% of the Federal Poverty Level, this couple was shielded under the old rules but saw their annual net premium jump from \$7,032 to \$35,712—a 408% rate increase driven by the combined force of the 21.5% gross rate hike and the severe reduction in their net tax credit.

The Dynamics of Enrollment Attrition and Risk Pool Degradation

The abrupt premium increases triggered an immediate contraction in Pennie's active enrollment. Understanding this attrition requires analyzing the chronological progression of plan cancellations, which reveals a highly predictable decay curve dictated by federal grace-period regulations rather than administrative discrepancies.

During the 2026 Open Enrollment Period which concluded on January 31, 2026, Pennie recorded approximately 85,000 active plan terminations. Concurrently, new enrollees dropped to roughly 79,500, representing a 12% decrease in new sign-ups relative to 2025. Although the open enrollment cycle initially ran 11% ahead of the prior year's pace, the late-stage realization of the subsidy cliff caused a rapid reversal, leaving Pennie with ~486,000 enrollees at the close of the period, which was 2% below the 2025 baseline.

By early April 2026, media and state reports indicated that cumulative terminations had reached approximately 120,000. By June 1, 2026, active enrollment had fallen further to 443,024, with cumulative cancellations exceeding 160,000. This sequential rise from 85,000 to 120,000 and ultimately to 160,000+ represents a continuous attrition timeline governed by two primary

mechanisms:

- **Binder Payment Defaults:** New marketplace applicants who failed to submit their initial premium payment within 30 days of their January 1 effective date never effectuated coverage. These non-effectuated enrollments are primarily captured in the initial 85,000 open enrollment drop.
- **Federal Grace-Period Expirations:** Returning enrollees receiving advanced premium tax credits are legally protected by a mandatory three-consecutive-month grace period under federal regulations. Insurers must pay clinical claims during the first month and are permitted to pend claims during the second and third months. For individuals who stopped paying premiums on January 1, 2026, this statutory grace period expired on March 31, 2026. Consequently, insurers were legally blocked from finalizing these terminations until April 1, which explains the sharp surge in cumulative cancellations reported in early April and throughout the second quarter.

Health policy researchers make the same point about official data, noting that standard industry snapshot reports historically released in July reflect February enrollment measured in mid-March. This timing means early reports are issued before many enrollees' grace periods have closed, understating the true toll. A full accounting is not expected until the full-year effectuated enrollment report is issued in summer 2027.

Rather than dropping coverage entirely, many financially strained enrollees executed a tier shift, migrating from Silver plans down to lower-premium Bronze plans. More than 33,000 Pennsylvania enrollees executed this switch for the 2026 plan year, representing a 30% jump in Bronze-tier enrollment compared to 2025. While this tier shift succeeds in reducing monthly premium outlays, it exposes consumers to significantly higher out-of-pocket medical liabilities. Actuarial data for 2026 highlights a severe deductible gap between the tiers, with the average marketplace deductible standing at \$5,304 for a Silver plan versus \$7,186 for a Bronze plan. This premium-for-deductible trade-off carries negative consequences for both consumers and health systems. In rural and medically underserved rating areas, underinsured consumers holding high-deductible Bronze plans frequently defer primary and preventative care, ultimately presenting at clinical facilities with advanced, higher-cost pathologies. This dynamic leads to high clinical default rates and a spike in uncompensated hospital care, placing financial strain on regional hospital systems. Furthermore, as healthier individuals drop coverage and sicker individuals concentrate in the remaining pool, carrier-level risk pools degrade. This morbidity degradation forces carriers to seek even higher rate increases in subsequent filing cycles, as seen in Ambetter's upwardly revised 37.8% approved rate increase for 2026.

Comparative Regional Analysis: The Rating Area Stress Scoring Model

To understand how these macroeconomic shocks propagate across the Commonwealth, health economists have developed a multidimensional Rating Area Stress Scoring Model. This framework evaluates each of Pennsylvania's nine rating areas across competitive density, enrollment attrition, network risk, clinical cost pressure, and marginal harm from the unfunded state subsidy. From this model, three structural market archetypes emerge inductively, illustrating why a single statewide premium statistic is a misleading policy indicator.

Rating Area & Hub	Counties Included	Attrition Archetype	Stress Score & Competitive Dynamics	Regional Carrier & Pricing Features
Area 8 (Philadelphia)	Bucks, Chester, Delaware, Montgomery, Philadelphia	Competitive Shock Absorption	6 / 10 (Low direct uninsured risk)	Dense 14-carrier market enabling consumer arbitrage ; Independence Blue Cross phasing out broad PPOs for tiered "Proactive" HMOs.
Area 3 (Scranton)	Lackawanna, Luzerne, Monroe, Wayne, Wyoming, others	Competitive Shock Absorption	7 / 10 (Moderate clinical cost pressure)	Wide premium spread ; Oscar's 23.1\% increase contrasted by Jefferson Health Plans' -10.1\% gross decrease.
Area 6 (Allentown)	Centre, Columbia, Lehigh, Northampton, Union, others	Vertically Integrated Stability	8 / 10 (High network disruption risk)	Bifurcated market ; Highmark's narrow "my Direct Blue Lehigh Valley EPO" competes with Geisinger and Jefferson HMOs.
Area 4 (Pittsburgh)	Allegheny, Armstrong, Butler, Fayette, Westmoreland, others	Vertically Integrated Stability	7 / 10 (Moderate morbidity risk)	Duopoly between Highmark (17.7\%) and UPMC Health Plan (20.1\%) ; clinical integration mitigates direct access gaps.
Area 5 (Altoona)	Bedford, Blair, Cambria, Clearfield, Somerset, others	Vertically Integrated Stability	8 / 10 (High uncompensated care risk)	Rural, mountainous footprint served by Geisinger, UPMC, and Highmark ; high Bronze tier migration.
Area 7 (York / Lancaster)	Adams, Berks, Lancaster, York	Vertically Integrated Stability	7 / 10 (Moderate subsidy cliff exposure)	Large self-employed population ; served by Keystone Central, Capital Advantage, Geisinger, and

Rating Area & Hub	Counties Included	Attrition Archetype	Stress Score & Competitive Dynamics	Regional Carrier & Pricing Features
				QCC.
Area 9 (Harrisburg)	Cumberland, Dauphin, Fulton, Juniata, Lebanon, Perry	Severe Exposure / Sole Remedy	9 / 10 (Extreme direct uninsured risk)	Thin carrier market ; Juniata and Fulton counties represent a causal failure where Pennie is the sole viable coverage vehicle.
Area 2 (Coudersport)	Cameron, Elk, Potter	Severe Exposure / Sole Remedy	10 / 10 (Critical market failure)	Near-monopoly served by Geisinger and Highmark ; characterized by an 87\% directory inaccuracy rate and aging enrollees.
Area 1 (Erie)	Clarion, Crawford, Erie, Forest, Warren, others	Vertically Integrated Stability	8 / 10 (High network adequacy risk)	Serve primarily by Highmark and UPMC ; plan choice narrows rapidly outside the Erie metropolitan area.

Competitive Shock Absorption (Rating Areas 8 and 3)

In high-density carrier markets, the premium shock caused by the subsidy cliff did not translate directly into immediate coverage loss. Rating Area 8 experienced the highest absolute enrollment loss in the Commonwealth—approximately 22,000 individuals—yet its 15.5\% proportional decline is lower than nearly every other rating area.

The resolution of this apparent paradox is carrier density: fourteen competing insurers meant that the premium shock translated to active plan migration rather than complete disenrollment for the consumers who could execute the switch. When Ambetter's 37.85\% rate increase became untenable, QCC's 15.18\% floor provided a viable migration target. Conversely, Independence Blue Cross is actively phasing out its broad PPO options in favor of tiered Proactive HMO networks, requiring enrollees to confirm that their specific specialists sit in the plan's top cost-sharing tier, rather than the network generally.

Similarly, in Rating Area 3, Partners Insurance Company (doing business as Jefferson Health Plans) offered a 10.12\% gross decrease, creating a strong premium anchor that allowed consumers to navigate away from Oscar's 23.12\% gross rate hike.

Vertically Integrated Stability with Morbidity Risk (Rating Areas 4, 5, 6,

and 7)

These markets are defined by large, clinically integrated health systems that pair proprietary insurance arms with direct ownership of hospitals and physician groups, such as Highmark and UPMC in Rating Area 4. Being insured and treated by the same system can facilitate smoother clinical referrals, but it also means that plan choice is effectively a hospital-network choice. UPMC's commercial plans carried some of the state's larger approved rate increases this cycle, with underwriters citing a sicker-than-projected commercial risk pool. While vertical integration provides regional pricing stability, it increases consumer vulnerability to network restructuring. This vulnerability is evident in Rating Area 6, where the historical stability of the Lehigh Valley market has been shattered by a bilateral contract dispute.

Severe Exposure / Sole Remedy (Rating Areas 9 and 2)

In the Commonwealth's most rural and carrier-thin rating areas, the premium shock represents an immediate driver of uninsured status. Rating Area 9's Juniata and Fulton counties represent a complete market failure; here, the unfunded state subsidy is not a contributing factor to uninsured status, but the direct proximate cause, because Pennie is the sole viable coverage mechanism and no carrier arbitrage exists to absorb the premium shock. Juniata County, for example, experienced a 485% surge in baseline premium costs for older, unsubsidized enrollees.

In Rating Area 2 (Cameron, Elk, and Potter counties), thin carrier competition, long drive times to specialists, and an older average enrollee age push baseline premiums higher before the subsidy cliff is even factored in. While Pennsylvania's Section 1332 reinsurance program offsets approximately 4% of what 2026 premiums would otherwise be, this buffer is modest against the scale of the federal subsidy loss. This market is further compromised by an 87% directory inaccuracy rate, rendering the search for in-network care highly difficult for the remaining enrollees.

The Lehigh Valley Crisis: The UnitedHealthcare and LVHN Network Fracture

In Rating Area 6 and Lehigh County, the commercial affordability crisis is exacerbated by a severe network disruption. Following a prolonged, two-year contract dispute, Lehigh Valley Health Network (now a wholly owned subsidiary of the 32-hospital Jefferson Health system) terminated its contract with UnitedHealthcare. This network fracture has unfolded across two distinct operational timelines :

- **The Medicare Advantage Disruption:** On January 26, 2026, Lehigh Valley Health Network officially exited UnitedHealthcare's Medicare Advantage network, affecting over 20,000 senior citizens in the region and causing approximately 5,400 active UnitedHealthcare Medicare Advantage policyholders to immediately lose in-network access to their established Lehigh Valley Health Network primary care physicians and specialists.
- **The Commercial and Employer-Plan Disruption:** On April 26, 2026, the contract covering UnitedHealthcare employer-sponsored commercial plans, individual marketplace plans, and the Veterans Affairs Community Care Network expired, displacing approximately 50,000 active commercial enrollees in the Lehigh Valley.

An important legal and operational exception exists regarding the Veterans Affairs Community Care Network. Although early communications suggested that Veterans enrolled in the UnitedHealthcare-administered Veterans Affairs Community Care Network would face the same out-of-network transition as commercial enrollees, subsequent joint statements from UnitedHealthcare and Lehigh Valley Health Network confirmed that Veterans are legally exempt. Veterans may continue to access all Lehigh Valley Health Network facilities and clinicians under the network without interruption. Similarly, UnitedHealthcare's state-administered Medicaid and Children's Health Insurance Program plans remain fully in network.

The contract breakdown is characterized by conflicting public narratives from both corporate entities :

- **UnitedHealthcare's Actuarial Argument:** UnitedHealthcare publicly positions Lehigh Valley Health Network as "one of the most expensive health systems in eastern Pennsylvania". Insurer executives allege that Lehigh Valley Health Network deliberately stalled contract negotiations for over six months, using the highly sensitive Medicare Annual Enrollment Period as leverage to demand an unsustainable, one-year price hike exceeding 20% for its commercial plans. To justify its resistance, UnitedHealthcare published localized price comparisons, asserting that routine diagnostic and clinical services at Lehigh Valley Health Network are priced significantly above regional averages

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 $\text{Excess CT/MRI Cost at LVHN} = +\$1,800 \quad \text{Excess ER Visit Cost} = +\$1,100$
 $\quad \text{Excess Total Hip Replacement Cost} = +\$24,000$

UnitedHealthcare argues that agreeing to these rates would force local employers to absorb massive premium increases, reducing capital available for employee wages and business expansion.

- **Lehigh Valley Health Network's Clinical Argument:** Conversely, Jefferson-LVHN leadership argues that UnitedHealthcare has consistently engaged in bad-faith negotiation tactics, refusing to provide reimbursement rates that reflect the inflationary costs of modern clinical delivery. Lehigh Valley Health Network alleges that UnitedHealthcare breached prior contractual covenants by unilaterally cutting clinical reimbursement rates by nearly 40% in 2021—an action currently being challenged through formal legal and judicial channels. Lehigh Valley Health Network asserts that UnitedHealthcare utilizes aggressive prior-authorization audits and excessive claim denials to delay payments and maximize corporate profitability at the expense of clinical care.

In response to the immediate panic surrounding the January Medicare Advantage termination, the Shapiro Administration intervened. Utilizing the state's regulatory authority, Pennsylvania Insurance Commissioner Michael Humphreys secured a federal Special Enrollment Period running from March 16 through April 30, 2026, specifically for UnitedHealthcare Medicare Advantage policyholders who utilized Lehigh Valley Health Network providers.

This regulatory intervention allowed displaced seniors to shop for alternative Medicare Advantage plans or transition back to traditional Original Medicare. Crucially, the Special Enrollment Period granted these seniors the right to purchase a Medicare Supplement policy without medical underwriting, protecting them from coverage denials based on pre-existing conditions.

The disruption also triggered federal political fallout, with U.S. Representative Rob Bresnahan, Jr. sending a formal letter to UnitedHealth Group and Jefferson Health leadership expressing deep frustration over the contract breakdown. Representative Bresnahan criticized both organizations for utilizing patients as negotiating leverage, highlighting the financial anxiety,

disruption to care, and confusion experienced by constituents across Northeastern Pennsylvania. The congressional intervention demanded that both parties immediately resume negotiations and provide formal updates on steps taken to mitigate the clinical impact on families.

Market Realignment: St. Luke's and the Main Line Health Contrast

The UnitedHealthcare-LVHN fracture has triggered a major realignment of healthcare market share in Lehigh County, directly benefiting Lehigh Valley Health Network's primary regional competitor, St. Luke's University Health Network.

Anticipating the contract termination, UnitedHealthcare executed a multi-year in-network agreement with St. Luke's University Health Network. This agreement covers UnitedHealthcare's commercial, Medicare Advantage, Medicaid, and CHIP plans. To capture the displaced patient volume, St. Luke's established a dedicated clinical scheduling hotline (484-526-4440) specifically for UnitedHealthcare members seeking to transition their primary and specialized care away from Lehigh Valley Health Network.

While this competitive alliance provides a theoretical safety valve for network adequacy, the physical capacity of the regional healthcare system has been severely strained. Local clinical reports indicate that the influx of thousands of displaced Lehigh Valley Health Network patients has overwhelmed St. Luke's administrative and clinical infrastructure.

For example, pediatric cardiology and other high-demand specialties at St. Luke's are fully booked through late 2027, with many departments closed to new patients. This capacity constraint highlights a critical policy failure: contractual network adequacy on paper does not translate to actual clinical capacity in practice. Displaced patients requiring complex, continuous specialty care face long wait times, presenting a risk to clinical outcomes.

This prolonged disruption in Lehigh County stands in stark contrast to a concurrent contract dispute in southeastern Pennsylvania between UnitedHealthcare and Radnor-based Main Line Health. Main Line Health—which operates Lankenau Medical Center, Bryn Mawr Hospital, Paoli Hospital, Riddle Hospital, Bryn Mawr Rehab Hospital, and Main Line HealthCare—was scheduled to go out of network for UnitedHealthcare commercial, ACA marketplace, and Medicare Advantage members on July 1, 2026, potentially impacting 32,000 patients.

However, on June 25, 2026, Main Line Health and UnitedHealthcare announced an agreement in principle. To finalize the multi-year contract terms, the parties extended active in-network access through July 14, 2026, ensuring no immediate disruption to patient care.

According to Main Line Health's billing disclosures, the new agreement successfully adjusted UnitedHealthcare's reimbursement rates to reflect the rising cost of care while establishing concrete administrative protocols to reduce prior-authorization delays, claim denials, and clinical audit friction. This eleventh-hour resolution highlights that regional health networks utilize different leverage strategies, with Lehigh Valley Health Network executing a complete split while Main Line successfully negotiated a last-minute compromise.

Statutory Protections, Consumer Recourses, and PBM Oversight

Lehigh County consumers caught in the crossfire of the premium shock and the

UnitedHealthcare-LVHN network split must navigate a complex landscape of state and federal regulatory protections. The primary legal shields include:

- **28 Pa. Code § 9.725 (Directory Accuracy Cap):** If an insurer's published provider directory contains an error—such as listing an out-of-network Lehigh Valley Health Network clinician as in-network—and a consumer relies on that directory to receive care, the consumer's financial liability is legally capped at standard in-network cost-sharing rates.
- **Act 252 of 2023 (Balance Billing Prohibition):** Under this state statute, any undisclosed out-of-network balance bill is legally presumed to be illegal. Providers are blocked from billing patients for the difference between the insurer's out-of-network reimbursement and the provider's gross charge unless explicit, advance consent was obtained.
- **Federal Continuity of Care Rules:** Under federal statutory guidelines, consumers who are actively undergoing a course of treatment for a serious and complex condition at the time of a network termination can petition UnitedHealthcare for transitional in-network coverage. This protection is not automatic and must be formally requested by the member and approved by UnitedHealthcare's clinical review board. If approved, UnitedHealthcare is required to cover the specific, ongoing treatment at in-network cost-sharing levels for up to 90 days, and longer in cases of active pregnancy through postpartum care. Qualifying clinical conditions include:
 1. Active pregnancy and postpartum care.
 2. Active cancer chemotherapy or radiation regimens.
 3. Scheduled non-elective major surgeries, including post-operative follow-up.
 4. Ongoing inpatient hospitalizations.
 5. Terminal illness requiring palliative care.

For services that do not meet these strict criteria, such as routine primary care, elective procedures, or annual diagnostic screenings, UnitedHealthcare is under no obligation to provide coverage. Members holding PPO plans can still access Lehigh Valley Health Network, but they will be subject to substantially higher out-of-network deductibles, higher coinsurance rates, and elevated out-of-pocket maximums. For those holding restrictive HMO or EPO plans, out-of-network care at Lehigh Valley Health Network is completely uncovered, leaving the patient personally liable for 100% of the clinical bill.

- **Federal Independent Dispute Resolution Misconceptions (45 C.F.R. § 149.510):** Policy researchers emphasize that many consumers are misled into believing they can file a federal Independent Dispute Resolution claim under the No Surprises Act to resolve billing disputes. This process is strictly a provider-versus-plan arbitration mechanism designed to resolve out-of-network payment rates between insurers and clinicians. Individual patients have no standing to initiate a dispute and must instead rely on state-level insurance department complaints (via 1-866-722-6675) or the free legal advocacy provided by the Pennsylvania Health Law Project (1-800-274-3258).
- **Pharmacy Benefit Manager Oversight (Act 77 of 2024):** Beyond medical network architecture, the Pennsylvania Insurance Department is enforcing new regulatory milestones under Act 77 of 2024 to increase transparency in the prescription drug supply chain. Pharmacy Benefit Managers are facing strict reporting deadlines in 2026: the first PBM Network Adequacy Report was due on April 1, 2026, and the first PBM Transparency Report is due on July 1, 2026. Regulators are legally mandated to post these transparency reports publicly within 60 days of receipt, exposing spread pricing, manufacturer rebate structures, and regional pharmacy access restrictions to public scrutiny.

The Legislative Budgetary Stalemate and the Future Outlook

The ultimate resolution to the individual market affordability crisis rests in the hands of the Pennsylvania General Assembly. Act 54 of 2024 officially established the State Health Insurance Exchange Affordability Program—a state-level premium subsidy designed specifically to assist households earning between 151\% and 300\% of the Federal Poverty Level. According to actuarial modeling from Pennie, a state investment of \\$50 million—representing a mere 1/12th of the cost of the expired federal EPTC program—would deliver immediate relief :

- It would reduce average monthly premiums by 9\% to 12\% for eligible participants.
- It would provide direct financial support to over 280,000 active and enrolling Pennsylvanians, allowing them to buy up to higher-value Silver or Gold plans.
- It would provide a critical safety net that would enable nearly one-fourth of the 160,000+ individuals who dropped coverage to return to the exchange.

Although Pennie administrators have repeatedly stated that the administrative infrastructure is fully developed and ready to deploy the subsidies the moment funding is authorized, the program remains entirely unfunded. This delay is due to the General Assembly missing its constitutional June 30 budget deadline.

As of July 4, 2026, Governor Josh Shapiro and legislative leaders remain locked in closed-door negotiations over a package worth more than \\$53 billion. The budget delay is a structural pattern in Harrisburg, marking the fifth consecutive year that lawmakers have blown past the statutory deadline.

The Republican-controlled Senate adjourned over the Independence Day holiday weekend without a deal, though leadership from both parties expressed optimism that a final package would be finalized in the days following July 4.

At the core of the current FY 2026-27 budget stalemate is a deep ideological divide regarding revenue generation, tax structures, and education policy. Governor Shapiro's proposed \\$137.2 billion combined operating budget calls for \\$53.3 billion in state General Fund expenditures, which represents a 5.38\% increase. To balance the proposed spending, the executive budget relies on several highly contested fiscal bridging measures :

- **Rainy Day Fund Drawdown:** The proposal transfers \\$4.58 billion out of the Budget Stabilization Reserve Fund (Rainy Day Fund) directly into the General Fund.
- **New Revenue Sourcing:** The budget seeks \\$1.88 billion in new tax revenue, primarily generated by taxing regulated skill games, legalizing and taxing adult-use recreational marijuana, and implementing a statewide minimum wage increase to \\$15 per hour.
- **Delaware Loophole Closure:** The executive budget proposes closing the corporate net income tax loophole that permits large companies to shift Pennsylvania-based income to out-of-state subsidiaries.
- **Innovate in PA 2.0:** The administration proposes \\$100 million in Insurance Premium tax credits to provide capital for venture funds and competitive grants, projecting a revenue impact of \\$7.5 million in FY 2028-29.

These revenue measures are tied directly to healthcare and social safety net programs. For example, the proposed \\$15-per-hour minimum wage increase is projected to reduce the number of Pennsylvanians qualifying for Medicaid by approximately 60,800. The executive

budget notes that children in these impacted families would instead transition to the Children's Health Insurance Program, which is slated for a \$7.6 million funding increase to cover the expanding enrollment and adjust reimbursement rates for CHIP insurance carriers.

Furthermore, the budget includes \$1 million to implement the housing components of the state's newly approved Keystones of Health Medicaid demonstration waiver, alongside \$900,000 for the Food is Medicine nutritional initiative.

Outside of healthcare, the budget's progress remains blocked by a bitter dispute over public versus private education. House Democrats are prioritizing a \$565 million adequacy supplement distributed through the Ready to Learn Block Grant to target underfunded school districts.

In contrast, Senate Republicans are demanding an expansion of the Educational Improvement and Opportunity Scholarship Tax Credit programs, currently capped at \$575 million.

Republicans passed a bill expanding the scholarship credits by \$25 million, while House Democrats passed a competing bill that would implement strict accountability standards, requiring compliance audits by the state's auditor general and a 2% administrative fee on contributions to cover operational costs.

Other unresolved budget items include the Low-Income Home Energy Assistance Program, which provides cash heating grants ranging from \$200 to \$1,000 to vulnerable families, and a legislative push to expand Act 54 of 2024's municipal taxing authority. The proposed municipal tax bill would permit second-class townships to raise local tax limits via ordinance to fund essential staffing, training, and equipment costs for local firefighters and EMS companies.

Because the Pennsylvania Supreme Court ruled in 2009 that the federal Fair Labor Standards Act mandates the continued payment of state employee salaries during an impasse, the immediate operational pressure on lawmakers to pass a budget has been eliminated.

However, as the budget delay enters the second week of July, the suspension of discretionary state payments will begin to strain school districts, county-level mental health networks, early childhood intervention providers, and local non-profits, many of which must spend down cash reserves or take out high-interest bridge loans to survive.

For the 160,000 uninsured Pennsylvanians and the thousands of displaced patients in Lehigh County, this legislative gridlock represents a silent driver of clinical and financial vulnerability.

Until a compromise is signed, the individual health insurance market in Pennsylvania will remain highly volatile, with regional networks operating under severe strain.

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